SALES AND MEDICAL

Case Studies

ELIGIBLE DISCIPLINES

Open to pre-final and final-year students pursuing a full-time MBA/PGDM with a specialization in Sales & Marketing or Pharmaceutical Management.



CASE STUDY: SALES TRACK

Selling Smarter: Building a High-Impact Sales Engine with Reduced Waste

01 CONTEXT

India's beauty and personal care market operates across general trade, modern trade, e-commerce, quick commerce, and salons. On-ground Sales teams are responsible for driving revenue, but also play a critical role in ensuring products move to consumers once they reach distributors and retailers. When products don't move as expected, they can age in the channel, get returned due to damage, or expire - a cost that hits both the P&L and sustainability goals.

02 DATA POINTS

~1.5%

Return/expired/damaged stock is capped as ~1.5% of total revenue

~1%

The broader cost of managing this (handling, reverse logistics, destruction) is estimated at ~1% of revenue

- **Field sales teams have direct visibility into distributor and retailer inventory, product velocity, and local demand signals**

Students are encouraged to draw on publicly available industry reports, retail observations, and field-sales frameworks to support their answers.

CASE STUDY: SALES TRACK

03 CHALLENGE

You are part of the on-ground Sales team of a leading beauty organization. Management wants Sales to protect revenue growth while proactively reducing the stock that ends up as returns, expiry, or dead inventory without simply cutting orders and risking stock-outs or lost sales. Design an on-ground sales strategy that drives market growth while catching slow-moving stock early enough to act on it before it becomes a write-off.

04 KEY DELIVERABLES

a Early Detection

- At which points during a retailer/distributor visits can slow-moving or ageing stock be spotted?
- What simple signals (e.g., shelf dust, unsold cartons, retailer feedback) should be flagged, and to whom?

b On-Ground Intervention

- What specific on-ground actions would you use to move slow SKUs?
- What can we do differently for a near expiry SKU versus a slow-moving one?

CASE STUDY: MEDICAL TRACK

Engaging Dispensing Dermatologists

01 CONTEXT

In many markets, a significant share of dermatologists don't just prescribe products -they also stock and directly sell them, acting as a hybrid medical-and-retail channel. This "dispensing" model changes the nature of the relationship: these doctors aren't just being asked to trust and recommend a product, they're being asked to actively invest in, stock, and sell it - much like a retailer would.

Your brand is piloting a refill program for one of its dermatologist-recommended ranges, aimed at reducing packaging waste for patients who use the product daily over long periods. For this refill program to work at all, it depends entirely on consistent, correct execution inside the doctor's own clinic - something the brand cannot directly control the way it might in a company-owned retail environment.

02 DATA POINTS

~45%

Approximately 45% of dermatologists in the market operate a dispensing model, stocking and selling products directly to patients from their clinics.

- **A refill program (patients returning empty packaging in exchange for a discounted refill) is a common sustainability mechanic in retail, but is largely untested in a clinic-dispensing environment.**

Applicants are encouraged to draw on publicly available information on dermatologist dispensing practices, refill/circular packaging models in retail, and general assumptions about clinic operations to support their answers.

CASE STUDY: MEDICAL TRACK

03 CHALLENGE

You are part of the Medical team responsible for engaging dermatologists. Your task is to design an approach that wins over dispensing dermatologists as a segment (different from purely prescribing doctors) and gets the refill sustainability program reliably executed within their clinic's own dispensing operation.

04 KEY DELIVERABLES

a Understanding the Dispensing Dermatologist

- How is a dispensing dermatologist's decision-making different from a purely prescribing dermatologist?
- What would motivate the doctor to actively support a refill program?

b Getting the Refill Program Executed In-Clinic

- What specific levers (training, incentives, tools, or simplified processes) would you use to ensure execution of the refill program?

c Making the Case to the Doctor

- What would your core pitch be to convince a dispensing dermatologist to prioritize stocking and promoting this range over other brands?
- How would you frame the sustainability angle so that it feels like a value-add for the doctor's practice and patients?

d Measuring & Sustaining Adoption

- What would you track to know whether the refill program is being executed consistently across your assigned territory?